



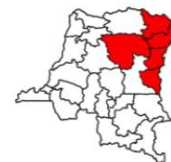
EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°107/MVEBDB/08/29/2026

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HIGHLIGHTS (last 24 hours)

For August 29, 2026, 96 new confirmed cases of Ebola Virus Disease (EVD), including 41 community deaths, were recorded in the provinces of Ituri (61 cases), North Kivu (28 cases), and Haut-Uélé (7). In addition, 8 Intra-CTE deaths have been recorded. No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 6041	ACCUMULATIVE DEATH CONFIRMED 2,911	LETHALITY 48.2%	PATIENTS IN ISOLATION/CTE 619	ACCUMULATIVE HEALED 1,366	TRACKING RATES CONTACTS 82.4% (Of the Day)
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo & Bas Uélé



60 out of 151 health zones affected (39.7%)



Summary of key points from the last 24 hours

In total, **96 new confirmed cases** including **41 community deaths** of Ebola Virus Disease (EVD) have been reported in the last 24 hours, in the provinces of **Ituri (61 cases), North Kivu (28), Haut Uélé (7)**.

Fortunately, **39 patients** were declared cured after receiving two negative Ebola virus (EV) test results (34 in Ituri and 5 in Haut-Uele). However, **8**

Intra-CTE deaths were recorded, including 7 in Ituri and 1 in Haut Uélé. This brings **the total number of deaths for the day to 49**.

From the start of the epidemic until August 29, 2026, **6,041 confirmed cases, including 2,911 deaths**, were reported in the DRC, representing a case fatality rate of **48.2 %**. Ituri remains the epicenter of the epidemic, accounting for **82.3%** of cumulative confirmed cases and **63.5%** of new reported cases.

Overall, 60 of the 151 (39.7%) health zones in the six provinces remain affected since the start of the epidemic in the DRC. Ituri has 28 health zones affected out of 36, followed by North Kivu (15/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas Uélé (3/11).

Table 1. Distribution of confirmed cases and deaths by affected province, as of August 29, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	61	4972	2,247	45.2%	28/36 (77.8%)
North Kivu	28	828	559	67.5%	15/34 (44.1%)
Haut-Uélé	7	216	94	43.5%	6/13 (46.1%)
Tshopo	0	19	8	42.1%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas Uélé	0	3	2	66.7%	3/11 (27.3%)
Total	96	6041	2,911	48.2%	60/151 (39.7%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of August 29, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	4972	2247	45.2%	61	28	7	35
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	158	35	22.2%	1	0		0
Boga	2	2	100.0%				0
Bunia	1392	437	31.4%	17	7		7
Damascus	28	9	32.1%	1	0		0
Drodro	13	6	46.2%				0
Fataki	74	30	40.5%				0
Gethy	6	2	33.3%				0
Kambala	2	0	0.0%				0
Kilo	32	12	37.5%				0
Komanda	74	48	64.9%	2	0		0
Lita	218	119	54.6%	8	8		8
Logo	11	5	45.5%				0
Lolwa	19	4	21.1%				0
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	36	21	58.3%				0
Mangala	236	122	51.7%	9	5		5
Mongbwalu	609	285	46.8%	1	0		0
Nia-Nia	214	113	52.8%	9	5		5
Nizi	652	260	39.9%	6	2		2
Nyankunde	124	35	28.2%				0
Rimba	10	4	40.0%				0

Rwampara	945	337	35.7%	6	1		1
Tchomia	57	25	43.9%	1	0		0
To ventilate	N/A	314	N/A	N/A	N/A	7	7
North Kivu	828	559	67.5%	28	13	0	13
Blessed	136	98	72.1%	8	5		5
Good	1	1	100.0%				0
Butembo	164	124	75.6%	4	3		3
Goma	1	0	0.0%				0
Kalunguta	16	7	43.8%				0
Katwa	389	256	65.8%	12	3		3
Kyondo	16	12	75.0%				0
Lubero	1	1	100.0%				0
Mabalako	3	2	66.7%				0
Manguredjipa	1	0	0.0%				0
Masereka	8	4	50.0%				0
Musienene	78	45	57.7%	3	2		2
Mutwanga	1	1	100.0%				0
Oicha	7	6	85.7%				0
Vuhovi	6	2	33.3%	1	0		0
Haut-Uélé	216	94	43.5%	7	0	1	1
Boma Mangbetu	27	14	51.9%	1	0		0
Gombari	3	1	33.3%				0
Isiro	81	33	40.7%	2	0		0
Pawa	28	16	57.1%	4	0		0
Rungu	1	0	0.0%				0
Wamba	76	30	39.5%			1	1
Tshopo	19	8	42.1%	0	0	0	0
Bafwasende	2	0	0.0%				0
Kabondo	3	1	33.3%				0
Lubunga	1	0	0.0%				0
Makiso- Kisangani	9	4	44.4%				0
Mangobo	2	2	100.0%				0
Tshopo	1	0	0.0%				
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	3	2	66.7%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	1	0	0.0%				0
Ganga	1	1	100.0%				0
Total	6041	2911	48.2%	96	41	8	49

*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

** These deaths were recorded in the various CTEs of the Ituri province and are currently being assessed.

Table 3. Status of alerts notified by province, as of August 29, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	17	2	19	12	2	0	0	14	12
Bas Uélé	13	1	14	1	1	12	0	2	0
Ituri	680	40	720	106	33	325	6	139	90
North Kivu	899	52	951	105	52	637	0	157	77
South Kivu	0	0	0	0	0	0	0	0	0
Tshopo	126	0	126	8	0	116	0	8	2
Total General	1735	95	1,830	232	88	1090	6	320	181

ND: Not available.

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

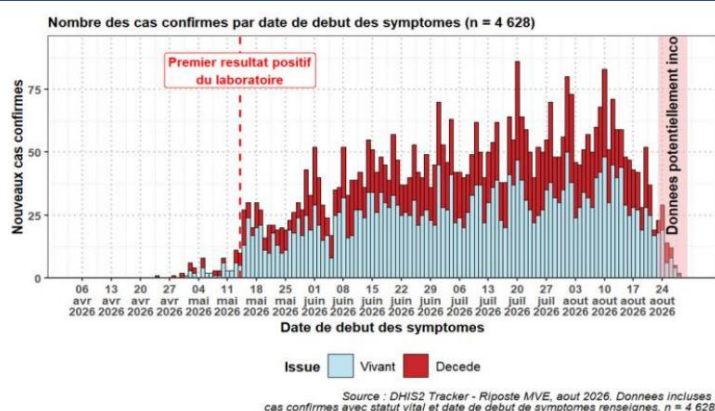
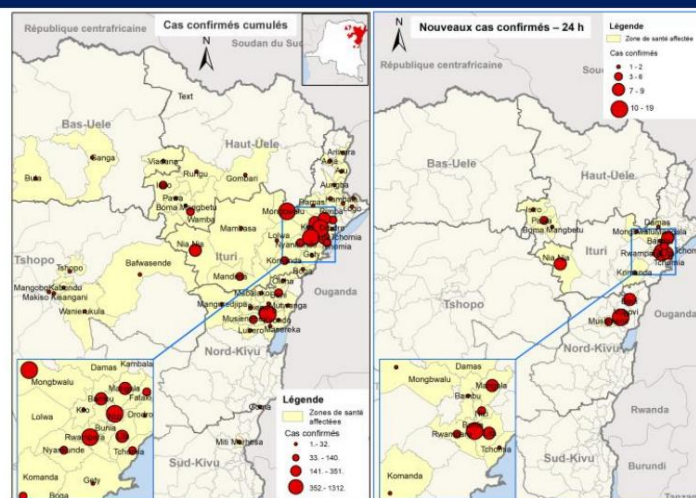


Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

1.1.2. Epidemiological surveillance

1.1.3. Main Actions

By the end of the day on August 29, 2026, 1,830 alerts had been recorded in the 5 out of the 6 currently affected areas, of which 1,416 (77.4%) were verified. All 320 confirmed suspected cases were investigated (100%), and 181 of them were transferred to CT/CTE (Table 3).

Over the past 24 hours, the proportion of contacts followed is at 82.4% (18,520 seen out of 22,463 to follow), below the threshold of 85%: Ituri 86.8% (9,770/11,270), Tshopo 44% (200/455), North Kivu 83.6% (7,525/8,996), Haut Uélé 57.4% (830/1,445) and Bas Uélé 62.6% (186/297).

1.1.4. Challenges

There is persistent low contact tracing around confirmed cases in the provinces of Bas and Haut Uélé, due to insufficient teams dedicated to these activities.

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

- Eight (8) alerts were notified to PoE/PoC in the last 24 hours: five in North Kivu, two of which were validated at PoC Mukulia (ZS Beni) and referred to the transit center and three invalidated after investigation; one in Haut-Uélé, at PoC Magambe (ZS Isiro), validated but the person concerned refused their transfer to the CT; two in Bas-Uélé, at PoC PK65 Sukisa and PK25 Koteli (ZS Koteli), both invalidated after investigation.

Table 4. Status of key PoC/PoE indicators by province, as of August 29, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Bas-Uélé	Overall
Report completeness	95% (57/60)	100% (18/18)	55.5% (5/9)	71.4% (5/7)	64% (7/11)	45% (9/20)	80.8% (101/125)
% of strategic PoE/PoC functional	9.09% (1/11)	100%	N / A	35% (7/20)	64% (7/11)	ND	ND
Number of people who have switched to PoE/PoC	146,033	85,571	8,267	6,691	13,719	4,412	264,693
% of travelers screened	97.5%	97.0% 98.0%	99.0% 99.0%	98.9%			97.5%
% of travelers who washed the hands	97.3%	97.0% 98.0%	99.0% 98.0%	98.9%			97.3%
% of travelers made aware	99.5%	97.0% 100%	98.0% 98.0%	98.9%			98.6%
Number of alerts notified	0	5	0	0	1	2	8
Number of live alerts validated (suspected cases)	0	2	0	0	1	0	3
Number of bodies recovered today	0	0	0	0	0	0	0
% of suspected cases transferred at CT/CTE	NA (0/0)	100% (2/2)	N / A	N / A	0% (0/1)	N / A	66.7% (2/3)
% of lifeless bodies swabbed/secured	NA (0/0)	NA (0/0)	N / A	N / A	NA (0/0)	N / A	NA (0/0)
% of suspected cases investigated within 24 hours	NA (0/0)	100% (2/2)	N / A	N / A	100% (1/1)	100% (2/2)	100% (5/5)

1.2.2. Challenges

Operationalization of PoE/PoC is suboptimal, and reporting completeness is insufficient (101 out of 125 reports: 80.8%), with particularly low levels in Bas-Uélé (45%), Sud-Kivu (55.5%), Haut-Uélé (64%), and Tshopo (71.4%). In Ituri, only 1 (9.09%) of the 11 strategic PoE/PoCs is operating without interruption, and the PoCs in Avakubi, Mabakese, and Apinaka have not reported. In Tshopo, 7 of the 20 planned PoCs are activated (35%).

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 61 new positive results** (33 living and 28 dead) out of 342 new samples received and analyzed (**positivity: 17.8 %**).
- **Tshopo:** 6 new samples were received, all came back negative.

- **North Kivu : 28 new positive results** (15 living and 13 dead) out of 157 samples analyzed (**positivity of 17.8%**).
- **Haut-Uélé: 7 new positive results** (7 live) out of 31 samples analyzed (**positivity: 22.6%**).
- **Bas-Uélé: 1 sample received and tested**, the result came back negative.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 33 rings were opened out of the 45 identified (73.3%), 117 households decontaminated out of 144 identified (81.3%) and 13 FOSA out of 14 (92.9%); 30 hygiene kits were provided to households in Mongbwalu and 4 FOSA assessments were carried out.
- **In Tshopo**, monitoring and support were carried out for 11 social and solidarity economy (SSE) organizations in 5 health zones (Makiso-Kisangani, Mangobo, Tshopo, Kabondo and Lubunga) during which 119 service providers were briefed on the implementation of infection prevention and control (IPC) measures.
- **In Haut-Uélé**, the basic PCI training of 50 service providers in the Pawa health zone was completed; 2 rings were opened, with 5 and 6 households decontaminated out of 20 in Wamba.

1.4.2. Challenges

- Poor implementation of infection prevention and control (IPC) standards: in Ituri, the opening of infection control rings was suboptimal (73.3%) and household decontamination was only 81.3% complete; there was an insufficient number of decontamination teams in Fataki, Mangala, and Nizi, and a lack of mobility in Komanda, Bambu, Mandima, and Mangala. In North Kivu, only 4 rings were opened and 9 households were decontaminated out of 17 identified (52.9%).
- In Tshopo, insufficient PPE in ESS, untrained service providers and breakdown of the incinerator at the Makiso-Kisangani General Referral Hospital.

1.5. Continuity of care

1.5.1. Main actions

- **In Ituri**, 117 new admissions were recorded and 119 discharges, including 34 recoveries. Thus, 550 patients are hospitalized in 978 beds, representing an occupancy rate of 56.2%.
- **In Haut Uélé**, 12 new admissions of suspected cases were recorded and 9 discharges, including 5 recoveries; 44 patients are hospitalized out of 120 beds, representing an occupancy rate of 36.7 %.
- **In Tshopo**, 1 new admission was recorded compared to 4 discharges, and 10 patients are in isolation for 25 beds, i.e. an overall occupancy rate of 40.0%.
- **In South Kivu**, 15 suspected cases remain under hospital monitoring.

1.5.2. Challenges

Low capacity and absence of CTE in some health zones: in Haut-Uélé at the CTEs of Isiro and Wamba; in Ituri, the CTEs of Bambu and Nizi are saturated; in Bas-Uélé, absence of CTEs meeting standards and of a partner to support care.

1.6. Risk communication and community engagement

1.6.1. Main actions

- **In Ituri**, 185 people were affected around the latest confirmed cases (89 in Nia-Nia/Alimasi, 37 in Lolwa/Mabangifo, 26 in Rwampara, 19 in Lolwa and 14 in Kilo/Itendey); 78 children sensitized during a mass session in Nia-Nia in preparation for the start of the school year; 1 ring opened and 7 EDS facilitated in Rwampara; 137 contacts listed in Rwampara, 68 in Nia-Nia and 37 in Kilo with the support of the CREC.

- **In Tshopo**, awareness-raising of 477 people (175 women, 140 men and 162 children) among internally displaced persons in Konga-Konga and Simisimi with the support of the NGO GOVA; briefing of 64 SUPRO in prelude to the vaccination campaign; 3 radio broadcasts and broadcast of spots (RNTC, Eben Ezer and Canal Orient); 1 rumor clarified.
- **In North Kivu**, raising awareness among Wazalendo about participating in response activities and Securing 3 PoCs (Mususa, Cugeki and Kyaghala).
- **In Haut-Uélé**, 1,082 people reached through mass awareness campaigns and 77 educational talks; community dialogues in Pawa (252 people) and Isiro, 41 home visits and development of a communication strategy with opinion leaders and motorcycle taxi drivers; in Bas-Uélé, awareness campaigns in local languages on RTF Likati radio and revitalization of the CODESA of the Lobi and Mboli AS.

1.6.2. Challenges

Persistence of challenges at community level: in Haut-Uélé, refusal to collect taxes by three PPLs and 3 escapes in Isiro.

1.7. Mental Health and Psychosocial Support (MHPSS)

1.7.1. Main actions

- **In Ituri**, 19 people benefited from SMSPS interventions related to the latest cases confirmed (Bunia 8, Rwampara 5, Nizi 3, Lita 2 and Komanda 1).
- **In North Kivu**, 25 of the 55 new confirmed and suspected cases benefited from SMSPS interventions; 86 results were announced out of 102, including 25 positive results; 62 households were supported; 98 PPLs were supported; 5 children were followed up in the community; 2 out of 4 recovered patients were reintegrated (50%); the 4 health zones that notified are covered (100%).
- **In Haut-Uélé**, psychological support for a bereaved family in Neisu (ZS Isiro); 15 laboratory results announced (11 in Isiro and 4 in Pawa) including 4 positive; 46 support persons and 21 PPL supported.

1.7.2. Challenges

- **In Haut-Uélé**, SMSPS coverage is insufficient. **In North Kivu**, only 4 health zones are covered by interventions around the latest cases, and there is a lack of childcare facilities in the Ebola Treatment Centers (ETCs). **In Ituri**, there is no coverage in Bambu, Nia-Nia, Mambasa, and Lolwa; coverage remains limited to 5 of the 11 health zones that have reported cases (45%).

1.8. Logistics

1.8.1. Main actions

In Tshopo, a computer kit for data management was received (funded by the US CDC/IRT); a Starlink kit was deployed in Bafwasende, and gowns were provided to vaccination teams; and the construction of isolation sites (Madula, Kabondo, and Konga-Konga) and the high-capacity Ebola Treatment Center (ETC) was monitored.

In North Kivu, the Mabalako health zone received supplies of medicines; work continued on the construction of the Matanda treatment center and nutritional supplies were packaged for Biena and Manguredjipa. In Haut-Uélé, work resumed on the installation of the Isiro treatment center and IPC/EDS supplies were delivered to the Rungu health zone.

1.9. Security

1.9.1. Main actions

- In North Kivu, security coverage of the PCI teams in the AS of Butsili, Mdrandele and Kanzunzulili as well as at the Nyakunde CS; accompaniment of the EDS team to the Bundji cemetery for the burial of 4 bodies of confirmed cases and for swabs at the Peregrin CH and the Don Divin CH; security supervision and assessment at the Kyaghala PoE and the Kitakandi PoC.

1.9.2. Challenges

Securing strategic points of presence (PoPs) remains incomplete: in Haut-Uélé, the body intercepted at the Magambe PoP (Isiro Health Zone) was released by security forces without being able to be swabé (a type of body). In North Kivu, insecurity persists in burial areas, and the road leading to the Bundji cemetery (Beni Health Zone) remains impassable.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions.

In Ituri, continued awareness-raising on the PSEAH of response actors and communities; display of messages and distribution of leaflets in the health zones of Bunia, Aru, Rwampara and Mongbwalu; in Tshopo, integration of PSEAH messages into the awareness-raising sessions of PPLs of health establishments in Makiso-Kisangani.

1.10.2. Challenges

- Still insufficient coverage of PSEA activities: 25% of the health zones in Ituri (9/36) know the reporting mechanism and only 2 partners (WHO and IMC) report their PSEA activities; lack of mapping of stakeholders; insufficient mass awareness materials; lack of means of transport and computer equipment for the pillar.

KEY MESSAGES

- Continued transmission of EVD, with 96 new cases including 41 community deaths and 8 intra-CTE recorded in the last 24 hours, bringing the national total to 6,041 confirmed cases and 2,911 deaths.
- Ituri remains the epicenter of the epidemic, concentrating 82.3% of cumulative confirmed cases and 63.5% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).
- Surveillance shows good performance (100%) of suspected cases investigated while the follow-up rate (82.4%) is below the threshold of 85.0%.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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